

CEPHALEXIN CAPSULE 500 mg

VICELXX-M

Description

Green opaque and white opaque capsule with "CEX 500" printed on one end and "HD" printed on the other end of capsule.

Composition

Cephalexin (as monohydrate) 500 mg/capsule

Actions and Pharmacology

Cephalexin inhibits bacterial cell wall synthesis, probably by acylation of the membrane bound enzyme transpeptidase. This prevents cross linkage of peptidoglycan chains which is necessary for bacterial cell wall strength and rigidity. Cell division and growth are also inhibited and lysis of susceptible bacteria frequently occurs. Cephalexin is readily absorbed after oral administration; food may delay absorption. About 80% or more of a dose is excreted in the urine in the first 6 hours. Little is excreted via biliary system and in breast milk.

Indications

Administered by mouth for the treatment of:

- Infections of upper respiratory tract caused by *Streptococci* and *Streptococcus pneumoniae*
- Infections of genitourinary tracts caused by *Escherichia coli*, *Klebsiella species* and *Proteus mirabilis*
- Infections of the skin caused by *Staphylococci* and *Streptococci*
- Infections of middle ear (otitis media) caused by *Haemophilus influenzae*, *Neisseria catarrhalis*, *Streptococci* and *Staphylococci*.
- Infections of bones and joints caused by *Proteus mirabilis* and *Staphylococcus aureus*.

Contraindications

Patients hypersensitive to penicillins, penicillin derivatives, penicillamine and cephalosporins may be hypersensitive to cephalexin.

Precautions/Warnings

- It should be used with caution in patients with history of gastrointestinal disease, especially ulcerative colitis, regional enteritis or antibiotic associated colitis.
- Care is necessary with patients with renal impairment.
- **Pregnancy** : Cephalexin crosses placenta. However, adverse effects on fetus have not been documented. Safety of this product for use during pregnancy has not been established.
- **Breast feeding** : Cephalexin is excreted in breast milk. Although problems in humans have not been documented, risk benefit must be considered.
- Patients taking cephalexin may give positive reaction to glucose test and Coombs test.
- Before cephalexin therapy is instituted, careful enquiry should be made concerning previous hypersensitivity reactions to cephalosporins and penicillin. Cephalosporin C derivatives should be given cautiously to penicillin sensitive patients. Serious acute hypersensitivity reactions may require epinephrine and other emergency measures.

Main Side/Adverse Effects

- Nausea, vomiting, diarrhoea and abdominal discomfort
- Allergic reactions such as skin rashes, urticaria, eosinophilia, angioedema
- Anaphylaxis
- Rises in liver enzymes
- Neutropenia
- Superinfection with resistant micro-organisms, particularly *Candida*
- Pseudomembranous colitis
- Hypoprothrombinemia (unusual bleeding or bruising)
- Renal dysfunction (decrease in urine output or decrease in urine concentrating ability)

Drug Interactions

Probenecid may decrease renal tubular secretion of cephalexin when used concurrently, resulting in increased and more prolonged cephalexin serum concentration and toxicity. There have been isolated reports of cephalexin decreasing the efficacy of oestrogen-containing oral contraceptives. The potential for increased nephrotoxic exists when cephalexin are used with other nephrotoxic medications, such as potent loop diuretics (e.g. frusemide, ethacrynic acid) especially in patients with pre-existing renal function impairment.

Overdosage

Clinical features

Anorexia, nausea, vomiting, abdominal discomfort, diarrhoea, headache, drowsiness, dizziness, hepatic and renal damage.

Treatment

Emesis or gastric lavage, peritoneal dialysis and haemodialysis if appropriate, symptomatic and supportive measures.

Dosage and Administration

Usual adult dose : Oral, 250-500 mg every 6 hours.

Skin and skin structure infections : Oral, 500 mg every 12 hours.

Usual adult prescribing limits : Up to 4 g a day.

Note: When daily doses greater than 4 g are required, parenteral cephalosporins should be considered.

Usual paediatric dose : Oral, 6.25 to 25 mg per kg of body weight daily every 6 hours.

Skin and skin structure infections : Oral, 12.5 to 50 mg per kg of body weight every 12 hours.

Note: Therapy should be continued for at least 10 days in group A beta haemolytic streptococcal infections to help prevent the occurrence of acute rheumatic fever or glomerulonephritis.

Patients with renal dysfunction will require a reduction in dose.

Storage : Store below 30°C. Protect from light.

Presentation/Packing : Blisters of 10 x10's

Marketing authorization holder and manufactured for:

HOVID Bhd., 121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

By: Kotra Pharma (M) Sdn. Bhd., No. 1, 2 & 3, Jalan TTC 12, Cheng Industrial Estate, 75250 Melaka, Melaka, Malaysia.

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